NCCP 00371 · VERSION 4

Letrozole · daily hormone therapy

Take letrozole once daily continuously. The prescriber confirms whether the aim is before surgery, after surgery, extended adjuvant treatment or advanced-disease control.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Letrozole: material risks

☐ Joint and muscle pain or stiffness

☐ Vaginal dryness and sexual symptoms

☐ Lipid / cardiovascular risk changes

☐ Hot flushes, sleep disturbance and fatigue

☐ Bone thinning and fractures

☐ Uncommon liver abnormalities or serious allergic reactions

The usual prescription is once daily, but severe liver impairment can require a different frequency. Use the frequency on your prescription and confirm any change with the prescriber or pharmacist. An already reduced or unusual schedule must be reviewed before it is changed. Confirmed menopause is needed for this monotherapy; any ovarian suppression has its own prescription and monitoring.

Individual benefit, alternatives and concerns

Agreed checks and next review

FBC, renal and liver profile; confirm menopause with hormone tests when uncertain. Assess fracture risk and baseline bone density, particularly for adjuvant AI treatment. The NICE adjuvant baseline-DXA recommendation has an exception for patients already receiving adjuvant bisphosphonates. FBC, renal / liver profile and lipids when clinically indicated. Review bone density during treatment and after treatment according to bone risk. Neoadjuvant response and surgical planning need explicit review milestones.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.20 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending